

Question 1

NOT COMPLETE

Case Study Screen 1 of 6

The nurse in the emergency department (ED) is caring for a 78-year-old female client.

Nurses' Notes

Emergency Department

1000: Client was brought to the ED by the client's adult child due to increased shortness of breath this morning. The adult child reports that the client has been running a fever for the past few days and has started to cough up greenish mucus and to complain of soreness throughout the body. Client was hospitalized for issues with atrial fibrillation 6 days ago. History of hypertension. Vital signs: T 101.1° F (38.4° C), P 92, RR 22, BP 152/86, pulse oximetry reading 94% on oxygen at 2 L/min via nasal cannula. On assessment, the client's breathing appears slightly labored, and coarse crackles (rales) are noted in the bilateral lung bases. Skin slightly cool to touch and pale pink in tone; pulses 3+ and irregular. Capillary refill is 3 seconds. Client is alert and oriented to person, place, and time. The adult child states, "Sometimes it seems like my parent is confused." Peripheral venous access device (VAD) placed in right forearm.

The following 6 questions refer to this clinical scenario: NURS 1021 Unit 3 Case Study 1.

Select the 4 client findings that require **immediate** follow-up.

- ☐ 1. vital signs
- ☐ 2. lung sounds
- ☐ 3. capillary refill
- ☐ 4. client orientation
- ☐ 5. radial pulse characteristics
- ☐ 6. characteristics of the cough

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CORRECT

Score: 4 / 4 points

Rationale:

The vital signs require follow-up: a fever of 101.1°F and a pulse oximetry reading of 94% despite supplemental oxygen both signal an active, worsening process. The lung sounds require follow-up: coarse bilateral crackles are consistent with pneumonia. The radial pulse characteristics require follow-up: an irregular pulse could reflect the client's known atrial fibrillation becoming unstable under physiologic stress. The characteristics of the cough require follow-up: greenish (purulent) sputum points to a bacterial respiratory infection. Capillary refill of 3 seconds is only borderline/minimally delayed and does not clearly require immediate follow-up. Client orientation is documented as intact (alert and oriented to person, place, and time) on this assessment, even though the adult child reports occasional confusion historically; the current objective finding is normal.

Question 2

NOT COMPLETE

Case Study Screen 2 of 6

The nurse in the emergency department (ED) is caring for a 78-year-old female client.

Nurses' Notes

Emergency Department

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➤ For each client finding below, click to specify if the finding is consistent with the disease process of pneumonia, a urinary tract infection (UTI), or influenza. Each finding may support more than 1 disease process.

Client Findings	Pneumonia	Urinary Tract Infection	Influenza
fever	☐	☐	☐
confusion	☐	☐	☐
body soreness	☐	☐	☐
cough and sputum	☐	☐	☐
shortness of breath	☐	☐	☐

Note: Each column must have at least 1 response option selected.

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Client Findings	Pneumonia	Urinary Tract Infection	Influenza
fever	☒	☒	☒
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body soreness	☐	☐	☒
cough and sputum	☒	☐	☐
shortness of breath	☒	☐	☒

Note: Each column must have at least 1 response option selected.

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CORRECT

Score: 9 / 9 points

Rationale:

Fever is a nonspecific sign of infection consistent with all 3 processes. Confusion in an older adult is a classic atypical presentation of a UTI and can also occur with pneumonia through hypoxia or a systemic inflammatory response, but is not a hallmark influenza finding. Body soreness (myalgia) is a hallmark influenza symptom and is not characteristic of pneumonia or a UTI. Cough with purulent (greenish) sputum is characteristic of bacterial pneumonia; influenza classically causes a dry cough, and a UTI does not cause cough. Shortness of breath is consistent with pneumonia (the primary respiratory process) and can occur with influenza (especially with complications), but is not a UTI finding.

Question 3

NOT COMPLETE

Case Study Screen 3 of 6

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Nurses' Notes

Emergency Department

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➤ Complete the following sentence by choosing from the list of options.

The client is at highest risk for developing

Select...▼

.

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➤ Complete the following sentence by choosing from the list of options.

The client is at highest risk for developing

dysrhythmias

 .

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CORRECT

Score: 1 / 1 point

Rationale:

The client has a recent history of atrial fibrillation, and the physiologic stress of an acute infection (fever, hypoxia, tachycardia) commonly destabilizes an existing arrhythmia or provokes a new one, making dysrhythmias the highest risk. Hypoxia, stroke, and pulmonary embolism are all plausible general risks for an acutely ill older adult, but none is as directly tied to this client's specific known cardiac history as a dysrhythmia is.

Question 4

NOT COMPLETE

Case Study Screen 4 of 6

The nurse in the emergency department (ED) is caring for a 78-year-old female client.

Nurses' Notes

Emergency Department

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1200:

Called to bedside by the adult child who states that the client "isn't acting right." On assessment, client is difficult to arouse, pale, and diaphoretic. Vital signs: P 112, RR 32, BP 90/62, pulse oximetry reading 91% on 2 L/min via nasal cannula.

The nurse has reviewed the Nurses' Notes from 1200.

- For each potential nursing intervention, click to specify whether the intervention is indicated or not indicated for the care of the client.

Potential Nursing Interventions	Indicated	Not Indicated
Prepare the client for defibrillation.	☐	☐
Place client in a semi-Fowler's position.	☐	☐
Request an order to increase the oxygen flow rate.	☐	☐
Request an order to insert an additional peripheral VAD.	☐	☐
Request an order to administer an intravenous fluid bolus.	☐	☐

Note: Each row must have only 1 response option selected.

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Question 4

COMPLETE

Case Study Screen 4 of 6

The nurse in the emergency department (ED) is caring for a 78-year-old female client.

Nurses' Notes

Emergency Department

- 1000:

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Note: Each row must have only 1 response option selected.

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CORRECT

Score: 5 / 5 points

Rationale:

Defibrillation is not indicated: it treats pulseless ventricular fibrillation/tachycardia, and this client has a pulse and is not in cardiac arrest. A semi-Fowler's position is indicated to ease the client's work of breathing. Increasing the oxygen flow rate is indicated given the client's hypoxia (91% despite 2 L/min). An additional VAD is indicated to support anticipated fluids and IV medications. An intravenous fluid bolus is indicated given the client's new hypotension (90/62), tachycardia, and signs suggestive of early septic shock.

Question 5

NOT COMPLETE

Case Study Screen 5 of 6

The nurse has reviewed the Orders from 1215.

➤ Click to highlight the orders that the nurse should consider a priority.

Orders

Orders

1215:

- insert an indwelling urethral catheter
- vancomycin 1 g, IV, every 12 hours
- computed tomography (CT) scan of the chest
- 0.9% sodium chloride (normal saline) 500 mL, IV, once
- laboratory tests: blood culture and sensitivity (C & S), complete blood count (CBC), arterial blood gas (ABG)

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Question 5

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CORRECT

Score: 3 / 3 points

Rationale:

The vancomycin, the normal saline bolus, and the blood culture/CBC/ABG labs form the core of a sepsis-bundle response to a client with a likely serious infection and new signs of shock: cultures should be drawn before (or essentially alongside) starting antibiotics, antibiotics should be given promptly, and fluids address the hypotension. The indwelling urethral catheter and the chest CT are reasonable orders but are not as time-critical as the sepsis-bundle interventions.

Question 6

NOT COMPLETE

Case Study Screen 6 of 6

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Nurses' Notes

Orders

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➤ For each assessment finding, click to specify if the finding indicates that the client's condition has improved, not changed, or worsened.

Assessment Findings	Improved	Not Changed	Worsened
pale skin tone	☐	☐	☐
respirations, 36	☐	☐	☐
blood pressure, 118/68	☐	☐	☐
pulse oximetry reading 91%	☐	☐	☐
client interacting with adult child at bedside	☐	☐	☐

Note: Each row must have only 1 response option selected.

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Question 6

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Case Study Screen 6 of 6

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Nurses' Notes

Orders

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CORRECT

Score: 5 / 5 points

Rationale:

Compared to the 1200 assessment (P 112, RR 32, BP 90/62, pulse oximetry 91%, difficult to arouse, pale, diaphoretic), the blood pressure of 118/68 is improved (no longer hypotensive), and the client interacting with the adult child at the bedside is improved (previously difficult to arouse). Pale skin tone is not changed (still pale, as before). The pulse oximetry reading of 91% is not changed (identical to the 1200 value). Respirations of 36 are worsened (up from 32), showing the client's respiratory status remains a concern even as circulation and mental status are trending in the right direction.

The nurse is caring for a client who is receiving a blood transfusion and states, "I feel chilled and am having back pain."

➤ Which of the following actions should the nurse take? **Select all that apply.**

- ☐ 1. Stop the transfusion.
- ☐ 2. Check the client's vital signs.
- ☐ 3. Notify the client's primary health care provider.
- ☐ 4. Return the blood and infusion tubing to the blood bank.
- ☐ 5. Infuse 5% dextrose in water through the intravenous catheter.
- ☐ 6. Administer a dose of an antiemetic prescribed p.r.n. to the client.

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CORRECT

Score: 4 / 4 points

Rationale:

Chills and back pain during a transfusion are classic signs of an acute transfusion reaction. The nurse should stop the transfusion immediately, check vital signs, notify the primary health care provider, and return the blood bag and tubing to the blood bank for reaction workup. The line should be kept open with 0.9% sodium chloride, not 5% dextrose in water (Option 5), which can cause hemolysis. An antiemetic (Option 6) does not address this client's presenting symptoms (chills and back pain, not nausea) and is not one of the immediate reaction-response actions.

The nurse is preparing a staff education program about total parenteral nutrition (TPN).

➤ Which of the following information should the nurse include? **Select all that apply.**

- ☐ 1. "The TPN intravenous tubing should be changed once a week."
- ☐ 2. "TPN can be administered through a peripherally inserted central catheter (PICC)."
- ☐ 3. "Clients receiving TPN should be weighed daily."
- ☐ 4. "An infusion pump is used to deliver TPN."
- ☐ 5. "Serum glucose levels should be monitored in clients receiving TPN."

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CORRECT

Score: 4 / 4 points

Rationale:

TPN can be safely infused through a PICC line, a form of central venous access appropriate for TPN's high osmolarity. Daily weights track fluid and nutritional status. An infusion pump ensures precise, controlled delivery. Serum glucose must be monitored because of TPN's high dextrose content and hyperglycemia risk. TPN tubing must be changed every 24 hours, not weekly (Option 1), because of the high infection risk from its glucose- and lipid-rich composition.

The nurse has taught a client with bipolar I disorder who is experiencing a manic episode and is receiving lithium.

➤ Which of the following statements by the client would indicate a correct understanding of the teaching?

- ☐ 1. "I will increase my oral fluid intake to 2 to 3 L daily while taking the medication."
- ☐ 2. "I will experience an improvement in my condition 5 weeks after starting the medication."
- ☐ 3. "I should decrease my intake of dietary sodium after starting the medication."
- ☐ 4. "I should limit time spent in a sauna to 1 hour weekly while taking the medication."

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CORRECT

Score: 1 / 1 point

Rationale:

Maintaining generous fluid intake (2 to 3 L daily) is essential lithium teaching, since dehydration increases lithium reabsorption and raises toxicity risk. Lithium's antimanic effect is generally expected within 1 to 3 weeks, not 5 (Option 2). Decreasing sodium intake (Option 3) is dangerous and backward: low sodium causes the kidneys to retain more lithium, increasing toxicity risk; sodium intake should stay consistent. Prolonged heat exposure and sweating, such as from a sauna (Option 4), promote dehydration and should generally be avoided or closely managed, not simply "limited to 1 hour weekly" as though that were safe.

The nurse has administered haloperidol to a client with schizophrenia who is agitated.

➤ Which of the following findings would require **immediate** follow-up?

- ☐ 1. continued lack of motivation
- ☐ 2. reports of muscle stiffness
- ☐ 3. inappropriate emotional expressions
- ☐ 4. difficulty focusing due to blurred vision

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CORRECT

Score: 1 / 1 point

Rationale:
Muscle stiffness after a high-potency typical antipsychotic like haloperidol raises concern for an acute extrapyramidal reaction (dystonia) or early neuroleptic malignant syndrome, both of which require immediate follow-up. Lack of motivation (Option 1) and inappropriate emotional expressions (Option 3) are negative/baseline symptoms of schizophrenia, not acute medication emergencies. Blurred vision (Option 4) is a common, less dangerous anticholinergic side effect.

The nurse is teaching a client who is receiving newly prescribed propylthiouracil.

➤ Which of the following information should the nurse include?

- ☐ 1. "Carry emergency identification with you listing your condition and medication regimen."
- ☐ 2. "The medication dose will need to be reduced if you develop agranulocytosis."
- ☐ 3. "You will experience weight loss if the medication is effective."
- ☐ 4. "Increase your daily intake of foods containing iodine."

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CORRECT

Score: 1 / 1 point

Rationale:
Propylthiouracil carries a risk of agranulocytosis, so carrying emergency identification helps ensure prompt recognition and evaluation (such as a CBC) if the client develops fever or signs of infection. Agranulocytosis requires stopping the medication, not reducing the dose (Option 2). Propylthiouracil treats hyperthyroidism; as it becomes effective and the metabolism normalizes, the client would be expected to gain weight or stabilize, not continue losing weight (Option 3). Iodine intake should generally be limited, not increased, in a client on antithyroid medication (Option 4).

The nurse is preparing to administer a beta blocker to a client.

➤ Which of the following would be a contraindication to administer the medication?

- ☐ 1. heart block
- ☐ 2. myocardial infarction (MI)
- ☐ 3. heart failure
- ☐ 4. angina pectoris

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CORRECT

Score: 1 / 1 point

Rationale:
Beta blockers are contraindicated in heart block, since they further slow atrioventricular conduction and can worsen the block. Beta blockers are actually standard, indicated therapy following an MI (Option 2) and for angina (Option 4), both of which reduce myocardial oxygen demand. Certain beta blockers are also indicated for stable chronic heart failure (Option 3); heart failure is not a blanket contraindication the way heart block is.

The nurse is planning a staff education program about informed consent.

➤ Which of the following information should the nurse include? **Select all that apply.**

- ☐ 1. "The main value of informed consent is for protection against lawsuits."
- ☐ 2. "Clients may withdraw consent after signing the informed consent form."
- ☐ 3. "Clients must sign the informed consent form before receiving preprocedural medication."
- ☐ 4. "Nurses witness the signing of the informed consent form to confirm that consent is voluntary."
- ☐ 5. "The signed consent form serves as evidence that the informed consent process has taken place."

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CORRECT

Score: 4 / 4 points

Rationale:

Clients retain the right to withdraw consent at any time, even after signing. Consent must be obtained before administering sedating preprocedural medication, so the client's capacity to consent is not impaired. The nurse witnessing the signature helps confirm the client is signing voluntarily and is who they say they are. The signed form documents that the informed consent process took place. The main value of informed consent is protecting the client's right to self-determination, not primarily shielding the institution from lawsuits (Option 1).

The nurse has taught a client who is receiving alendronate.

➤ Which of the following statements by the client would indicate a correct understanding of the teaching? **Select all that apply.**

- ☐ 1. "I will take alendronate a half hour before I eat breakfast."
- ☐ 2. "I should avoid weight-bearing exercises while taking alendronate."
- ☐ 3. "I should discontinue alendronate if I experience nausea or vomiting."
- ☐ 4. "I will need to remain in an upright position for 30 minutes after I take alendronate."
- ☐ 5. "I should notify my primary health care provider if I experience difficulty swallowing while taking alendronate."

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CORRECT

Score: 3 / 3 points

Rationale:

Alendronate must be taken on an empty stomach, at least 30 minutes before the first food, beverage, or other medication of the day, and the client must remain upright for at least 30 minutes afterward to prevent esophageal irritation. Difficulty swallowing should be reported promptly, since it can signal esophagitis or ulceration. Weight-bearing exercise (Option 2) should be encouraged, not avoided, to support bone density. Nausea or vomiting should be reported to the primary health care provider, not managed by self-discontinuing the medication (Option 3).

The nurse is developing a plan of care for a client with a spinal cord injury at C5 who has an indwelling urethral catheter.

➤ Which of the following would be a priority for the nurse to include in the plan of care?

- ☐ 1. encouraging the client to drink 6 to 8 glasses of fluid per day
- ☐ 2. maintaining the urine collection bag in a dependent position
- ☐ 3. teaching the client about foods high in fiber
- ☐ 4. assessing the color of the urine output

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CORRECT

Score: 1 / 1 point

Rationale:

Keeping the urine collection bag below the level of the bladder (a dependent position) is a fundamental catheter-care priority that prevents urine reflux and reduces infection risk, and applies regardless of other considerations. Adequate fluid intake (Option 1) supports urinary health but is a general goal rather than a catheter-specific priority. High-fiber foods (Option 3) address bowel, not bladder, management. Assessing urine color (Option 4) is reasonable general monitoring but is not as fundamental a safety priority as proper drainage positioning.

The nurse has been made aware that the following 4 clients require assistance.

➤ The nurse should **first** assist the client who had

- ☐ 1. an abdominal hysterectomy 5 hours ago and is reporting severe incisional pain
- ☐ 2. a transurethral resection of the prostate (TURP) yesterday and whose catheter has become disconnected
- ☐ 3. a lumbar laminectomy 2 days ago and is reporting that the feet are still numb
- ☐ 4. a spinal cord injury at T2 two weeks ago and is currently diaphoretic and nauseated

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CORRECT

Score: 1 / 1 point

Rationale:

New diaphoresis and nausea in a client with a spinal cord injury at T2 (above T6) are classic signs of autonomic dysreflexia, a life-threatening emergency that can rapidly progress to severe hypertension and stroke if the triggering stimulus (commonly bladder distension or bowel impaction) is not identified and relieved; this client must be seen first. Severe incisional pain (Option 1) needs treatment but is not immediately life-threatening. A disconnected catheter (Option 2) is a quick fix, not an emergency. Persistent numbness 2 days after a lumbar laminectomy (Option 3) needs evaluation but is not as acutely dangerous as untreated autonomic dysreflexia.

The nurse has taught a client who has a positive laboratory test result for human immunodeficiency virus (HIV) infection. The client is scheduled for a viral load test.

➤ Which of the following statements by the client would indicate a correct understanding of the teaching?

- ☐ 1. "The viral load test is used to determine my response to the treatment regimen I am receiving for HIV."
- ☐ 2. "The viral load test can rapidly detect HIV-specific antibodies in the blood."
- ☐ 3. "I will be able to decrease the dosage of my prescribed medications if my viral load is low."
- ☐ 4. "I am unlikely to develop acquired immune deficiency syndrome (AIDS) if my viral load is high."

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CORRECT

Score: 1 / 1 point

Rationale:

The viral load test measures the amount of HIV RNA in the blood and is used to monitor how well antiretroviral therapy is controlling the virus. It does not detect HIV-specific antibodies (Option 2), which is what an antibody/ELISA test does. Maintaining the full prescribed regimen, not reducing the dose, is what keeps the viral load low; reducing medication (Option 3) risks resistance and viral rebound. A high, not low, viral load (Option 4) reflects poor disease control and increases the risk of progression to AIDS.

The nurse is teaching a client who is scheduled for a 24-hour urine collection.

➤ Which of the following information should the nurse include? **Select all that apply.**

- ☐ 1. "You will be asked to urinate when starting the collection, and the initial urine will be discarded."
- ☐ 2. "A sign will be posted on the bathroom door as a reminder to save your urine."
- ☐ 3. "You will be asked to void at the end of the designated time period to complete the urine collection."
- ☐ 4. "You should discard urine that is dark or pink in color."
- ☐ 5. "The collected urine will be sent to the laboratory at the end of each shift."

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CORRECT

Score: 3 / 3 points

Rationale:

The standard protocol discards the first void to mark the true start time, saves every void afterward, posts a reminder sign, and includes the final void at the end of the collection period. All urine must be saved regardless of its color or appearance (Option 4); discarding any portion compromises the accuracy of the total collection. The complete collection is kept together (often on ice or refrigerated) and sent to the laboratory as one specimen at the end of the full 24 hours, not sent in portions at the end of each shift (Option 5).

The nurse has taught a client with diabetes mellitus (type 2) about foot care.

➤ Which of the following statements by the client would indicate a correct understanding of the teaching? **Select all that apply.**

- ☐ 1. "I will check my shoes for foreign objects prior to putting them on."
- ☐ 2. "I should use a large, coarse file to remove dry skin from a bunion."
- ☐ 3. "I will apply a petroleum-based ointment between my toes after bathing."
- ☐ 4. "I should avoid crossing my legs to prevent decreased circulation to my feet."
- ☐ 5. "I should wear new shoes for a few hours for several days until they fit well."

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CORRECT

Score: 3 / 3 points

Rationale:

Checking shoes for foreign objects protects against unnoticed injury given possible diabetic neuropathy. Avoiding crossing the legs prevents impaired circulation to the feet. Gradually breaking in new shoes over several days prevents blisters and injury. Aggressive self-filing of a bunion or callus with a coarse file (Option 2) risks skin breakdown; this should be done by a podiatrist. Ointment applied between the toes (Option 3) increases moisture and maceration risk in that area, promoting fungal infection; moisturizer should go on the tops and soles of the feet, not between the toes.

The nurse is teaching a client who is scheduled for a total hip arthroplasty via a posterior approach.

➤ Which of the following information should the nurse include? **Select all that apply.**

- ☐ 1. "The type of prosthesis used is based on the muscle strength and joint function of your upper extremities."
- ☐ 2. "Do not bend the affected hip more than 90 degrees after surgery."
- ☐ 3. "Skin preparation and cleansing is mandatory before surgery."
- ☐ 4. "Use an elevated toilet seat for at least 6 weeks after surgery."
- ☐ 5. "You can resume sexual intercourse after surgery if your partner is in a dependent position."

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CORRECT

Score: 3 / 3 points

Rationale:

Posterior-approach hip precautions include avoiding hip flexion beyond 90 degrees and using an elevated toilet seat to help maintain that precaution; pre-operative skin preparation and cleansing is standard surgical teaching. Prosthesis selection is based on the hip and lower-extremity anatomy and function, not the upper extremities (Option 1). Resuming intercourse safely depends on the client's own hip positioning to avoid excessive flexion, adduction, or internal rotation, not on the partner's position (Option 5).

The nurse is caring for a client who is receiving an intravenous infusion via a peripheral venous access device (VAD). The client reports sharp pain at the VAD site. The nurse notes the intravenous fluid is infusing more slowly than prescribed.

➤ The nurse should recognize that the client is **most** likely experiencing

- ☐ 1. venous spasm
- ☐ 2. nerve damage
- ☐ 3. septicemia
- ☐ 4. hematoma

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CORRECT

Score: 1 / 1 point

Rationale:

Venous spasm, a reflexive narrowing of the vein in response to irritation, classically causes both localized sharp pain and a reduced infusion rate because the vessel lumen narrows. Nerve damage (Option 2) would cause pain but would not mechanically slow the infusion. Septicemia (Option 3) would present with systemic signs such as fever and hypotension, not an isolated site finding. A hematoma (Option 4) classically presents with localized swelling and bruising rather than sharp pain as the primary symptom.

The nurse has attended a staff education program about obtaining blood specimens from a central venous access device (VAD).

➤ Which of the following statements by the nurse would require follow-up?

- ☐ 1. "I will use a 3 mL syringe to flush the catheter port."
- ☐ 2. "The injection cap should be cleansed with antiseptic and allowed to air-dry."
- ☐ 3. "I will aspirate 5 mL of blood and discard the syringe in the biohazard container before obtaining the specimen."
- ☐ 4. "The infusion should be turned off for at least 1 minute before the specimen is aspirated."

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CORRECT

Score: 1 / 1 point

Rationale:
A central VAD should never be flushed with a syringe smaller than 10 mL, since a smaller syringe generates higher pressure that can rupture or otherwise damage the catheter, so this statement requires follow-up. Cleansing and air-drying the injection cap, discarding an initial waste sample before collecting the actual specimen, and briefly pausing any infusion before aspirating are all correct, standard practices.

The nurse has taught about preventing osteoporosis to a 45-year-old client who has had a hysterectomy and bilateral salpingo-oophorectomy.

➤ Which of the following statements by the client would indicate correct understanding of the teaching?

- ☐ 1. "I will begin to take dancing lessons."
- ☐ 2. "I will get more rest at night."
- ☐ 3. "I will take a multivitamin supplement daily."
- ☐ 4. "I will add more fiber to my diet."

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CORRECT

Score: 1 / 1 point

Rationale:
Dancing is a weight-bearing exercise, and weight-bearing exercise stimulates bone formation and is a cornerstone of osteoporosis prevention, especially important after surgical menopause removes the protective effect of estrogen. Getting more rest (Option 2) and adding fiber (Option 4) are not targeted osteoporosis-prevention measures. A general multivitamin (Option 3) does not reliably provide the specific calcium and vitamin D intake that bone health requires.

The nurse is preparing to insert a peripheral venous access device (VAD) for a client.

➤ Which of the following actions should the nurse take?

- ☐ 1. Ask the client to open and close the fist multiple times.
- ☐ 2. Tap the client's vein multiple times to promote dilation.
- ☐ 3. Apply the tourniquet 9 to 10 in (22.5 to 25 cm) above the venipuncture site.
- ☐ 4. Palpate for a vein after cleansing the selected site.

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CORRECT

Score: 1 / 1 point

Rationale:

Light tapping over the vein promotes venodilation and is a simple, widely endorsed technique with no downside. Repetitive fist clenching (Option 1) is now discouraged by current practice standards because it can artificially alter certain lab values (such as potassium and lactate) if blood is drawn from that site. The tourniquet should be applied closer to the site, typically about 4 to 6 in (10 to 15 cm) above it (Option 3), not 9 to 10 in away. The vein should be selected and palpated before cleansing the site, not after (Option 4), since palpating after cleansing recontaminates the prepared site.

The nurse is assessing a newly admitted client who sustained partial-thickness (second-degree) burns to the anterior thorax in a house fire.

➤ Which of the following findings would require **immediate** follow-up?

- ☐ 1. dizziness and confusion
- ☐ 2. hypoactive bowel sounds and nausea
- ☐ 3. vesicular breath sounds throughout the lung fields
- ☐ 4. pain rated 5 on a scale of 0 (no pain) to 10 (severe pain)

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CORRECT

Score: 1 / 1 point

Rationale:

Dizziness and confusion in a client burned in a house fire are highly concerning for smoke inhalation injury, hypoxia, or carbon monoxide poisoning, and require immediate follow-up (assessing airway patency and obtaining a carboxyhemoglobin level). Hypoactive bowel sounds and nausea (Option 2) reflect an expected postburn stress ileus. Vesicular breath sounds (Option 3) are the normal breath sounds heard throughout most of the lung fields, not an abnormal finding. Moderate pain of 5/10 (Option 4) is expected with a burn injury and is manageable, not an emergency.

The nurse has taught a client with a hiatal hernia about interventions for the condition.

➤ Which of the following statements by the client would indicate a correct understanding of the teaching?

- ☐ 1. "I will consume 3 regular-sized meals daily."
- ☐ 2. "Wearing an abdominal binder can help relieve symptoms."
- ☐ 3. "I should elevate the head of the bed on 6 in (15 cm) blocks."
- ☐ 4. "Eating foods with a high fat content will increase gastric emptying."

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CORRECT

Score: 1 / 1 point

Rationale:

Elevating the head of the bed reduces nighttime reflux and is standard hiatal hernia/GERD teaching. Small, frequent meals are recommended instead of 3 regular-sized meals (Option 1), to reduce abdominal pressure. An abdominal binder (Option 2) increases intra-abdominal pressure and would worsen symptoms. High-fat foods (Option 4) delay, rather than increase, gastric emptying, which worsens reflux risk.

The nurse is assessing a client with suspected gout.

➤ Which of the following findings would support a diagnosis of gout? **Select all that apply.**

- ☐ 1. elevated serum uric acid level
- ☐ 2. a swollen, red joint
- ☐ 3. reports of moderate fatigue
- ☐ 4. distal extremities cool to touch
- ☐ 5. pain associated with movement of the affected extremity
- ☐ 6. intolerance of dairy products

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CORRECT

Score: 3 / 3 points

Rationale:

An elevated serum uric acid level (hyperuricemia), a swollen and red joint, and pain with movement of the affected joint are all classic gout findings, reflecting acute monoarticular inflammatory arthritis from urate crystal deposition. Fatigue is a nonspecific finding not characteristic of gout. Gout causes a warm, not cool, joint, so extremities cool to touch (Option 4) do not support the diagnosis. Dairy intolerance (Option 6) is unrelated to gout.

The nurse is assessing a client with suspected endometriosis.

➤ Which of the following findings would support a diagnosis of endometriosis?

- ☐ 1. dyspareunia
- ☐ 2. hot flashes
- ☐ 3. weight gain
- ☐ 4. amenorrhea

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CORRECT

Score: 1 / 1 point

Rationale:

Dyspareunia (painful intercourse) is a classic symptom of endometriosis. Hot flashes (Option 2) are associated with menopause, not endometriosis. Weight gain (Option 3) is not a characteristic endometriosis finding. Endometriosis is typically associated with dysmenorrhea and abnormal bleeding, not amenorrhea (absence of menses) (Option 4).

The nurse is assessing a client with cirrhosis.

➤ Which of the following findings would be consistent with a diagnosis of cirrhosis?

- ☐ 1. steatorrhea
- ☐ 2. deep vein thrombosis (DVT)
- ☐ 3. high fever
- ☐ 4. spontaneous bruising

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- ☐ 2. deep vein thrombosis (DVT)
- ☐ 3. high fever
- ☒ 4. spontaneous bruising

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CORRECT

Score: 1 / 1 point

Rationale:

Spontaneous bruising is a classic cirrhosis finding, resulting from decreased synthesis of clotting factors by the diseased liver. Steatorrhea (Option 1) is more characteristic of pancreatic insufficiency or malabsorption. DVT (Option 2) is not a hallmark cirrhosis finding. High fever (Option 3) is not a defining feature of cirrhosis itself, though it can occur with a complication such as spontaneous bacterial peritonitis.

The nurse is assessing a male client who has suspected syphilis.

➤ Which of the following findings would support a diagnosis of syphilis?

- ☐ 1. urethritis
- ☐ 2. conjunctivitis
- ☐ 3. chancre lesions
- ☐ 4. penile discharge

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CORRECT

Score: 1 / 1 point

Rationale:
A chancre, a painless genital ulcer, is the classic, pathognomonic finding of primary syphilis. Urethritis and penile discharge (Options 1 and 4) are more characteristic of gonorrhea or chlamydia. Conjunctivitis (Option 2) is not a classic syphilis finding.

The nurse has attended a staff education program about spinal shock following acute spinal cord injury.

➤ Follow-up is required if the nurse states that manifestations of spinal shock include

- ☐ 1. bowel dysfunction
- ☐ 2. bladder dysfunction
- ☐ 3. spastic paralysis below the level of injury
- ☐ 4. loss of sensation below the level of injury

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CORRECT

Score: 1 / 1 point

Rationale:
Spinal shock causes flaccid, not spastic, paralysis below the level of injury, along with areflexia; spasticity typically develops later, after spinal shock resolves. This statement is incorrect and requires follow-up. Bowel dysfunction, bladder dysfunction, and loss of sensation below the level of injury are all accurate, expected manifestations of spinal shock.

The nurse is planning care for a client who has an arteriovenous (AV) shunt in the left arm.

➤ Which of the following interventions should the nurse include in the client's plan of care?

- ☐ 1. Instruct the client to protect the AV shunt by tucking the left arm under the body while sleeping.
- ☐ 2. Check for a bruit by palpating the AV shunt.
- ☐ 3. Administer prescribed intravenous fluids through the AV shunt.
- ☐ 4. Avoid obtaining blood pressure measurements in the arm with the AV shunt.

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CORRECT

Score: 1 / 1 point

Rationale:

Blood pressure measurements, venipuncture, and IV access should never be performed in the extremity with an AV shunt or fistula, to protect it from pressure damage or occlusion. Tucking the arm under the body (Option 1) risks compressing and occluding the shunt. A bruit is assessed by auscultation (listening), while a thrill is assessed by palpation (feeling); "checking for a bruit by palpating" (Option 2) describes the wrong technique for the wrong finding. The AV shunt is reserved exclusively for dialysis access and should not be used for routine IV fluid administration (Option 3).

The nurse is caring for a client who has a chest tube attached to a closed-chest drainage system.

➤ It would be a priority for the nurse to monitor the client for

- ☐ 1. tracheal deviation
- ☐ 2. pain at the insertion site
- ☐ 3. subcutaneous emphysema
- ☐ 4. redness or swelling at the insertion site

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Rationale:
Tracheal deviation signals a tension pneumothorax and mediastinal shift, a life-threatening emergency that can rapidly cause cardiovascular collapse, making it the priority finding to monitor for. Pain at the insertion site (Option 2) is an expected, manageable finding. Subcutaneous emphysema (Option 3) can occur with a chest tube and needs monitoring but is generally not as immediately life-threatening as tracheal deviation. Redness or swelling at the insertion site (Option 4) may suggest local infection but is not an acute emergency.